

Prepare Fully Costed Budgets for Projects

Project Activities and Inputs

1. To develop a results-based budget, UNDP must plan and cost all activities and inputs to deliver the project output(s). Major activities and functions that must be accounted for within a project budget include both direct and indirect costs, as listed below and further detailed in Annex 1:

Direct Costs - Direct costs of programme, administrative and operational support activities, that are part of the project input¹:

- Programmatic activities (as listed in the project document, including goods and services)
- Project management
- Project communications, advocacy, and funding partner visibility
- Independent audit and Evaluation
- Quality Assurance (QA) services
- Monitoring, baseline data collection, surveys and evaluation of projects
- Project briefings and technical guidance for project stakeholders
- Project meetings, progress, and final reporting
- Donor-specific reporting
- Support to implementing and responsible parties
- HACT assessments and all assurance activities
- Project supervision and coordination (Page 5, #10 a. refers)
- Programme coordination (Page 5, #10 b. refers)
- Policy advice and Quality Assurance (Page 5, #10 c. refers)
- Risk management
- Activities leading to project closure (checklist)
- Administrative, operational, and other shared services
- Contingency

Indirect Cost – Costs that are additional to direct project costs, representing costs to the organization that are not directly attributable to specific projects or services, but necessary to fund the corporate structures, management and oversight costs of the organization. These are recovered by charging a cost recovery rate, known as General Management Support (GMS) fee.

¹ Within each applicable project activity/function, all necessary inputs such as personnel, delivery enabling services, equipment, travel, communication, and operating expenses should be budgeted.

2. While not all activities are applicable to all projects, for each of the above activities, all inputs must be budgeted in US dollars. Inputs must be based on internally available standard costs, such as pro forma rates, daily subsistence allowances, prevailing internal or market prices, and historical trends and events. Costs within a multi-year budget should be adjusted to reflect a reasonable estimate of anticipated inflation for the specific activities. Definitions of major budget inputs, such as personnel and operational costs are provided here.
3. UNDP utilizes the ability of personnel to create, obtain and apply knowledge to deliver development results in its programmes and projects. For the purposes of this guide, personnel will be referred to as all individuals contracted through staff and non-staff type contracts. Personnel represent significant input to programmes and projects, and feature prominently in programme and project budgets.
4. UNDP programmes and projects may draw upon personnel through different means, including contracting new personnel, receiving services from existing UNDP staff when full-time contracting within the project is not required, or sharing services of independent contractors when this is deemed more efficient. Project inputs likely include many such sources of personnel to achieve project results.

Budgeting for Personnel Costs

5. When planning personnel inputs, it is necessary to consider whether the function/activity needed to deliver the project requires personnel on a staff contract. Generally, there is a significant cost-differential between personnel on staff contracts versus other contractual modalities. Use of appropriate contractual modality should not be compromised solely to lower the required budget. Some functions require staff, while others can be conducted by non-staff personnel (such as consultants) for successful project delivery.
6. Personnel on a staff contract must be budgeted when the following are deemed necessary requirements for delivery: UNDP control functions; institutional knowledge of UNDP and the context in which it operates; strategic programme priorities that drive the project; organizational policies and procedures; consistency with UNDP position and quality assurance standards; exchange of UNDP knowledge and experience; and organizational risk management. Unless the project is large and long-term enough to justify contracting full-time staff, these services can typically be rendered by existing office staff, or staff in other cost centers.
7. Non-staff personnel should generally be contracted directly under the project. However, for certain common and shared costs (see detailed definitions of major budget inputs here), it may prove more efficient to share available office or other

cost center resources. In this case, a reasonable estimate of the anticipated share borne by the project must be discussed with the relevant operational teams. Note that budgeting of shared costs must be inclusive of personnel-related overhead, such as rent, communication, IT services, etc.

8. At the design stage, UNDP must transparently discuss each type of personnel requirement internally, as well as with donors and partners, and disclose the total budget for successful project implementation, in line with UNDP's financial, recruitment, procurement, and cost recovery policies.

Delivery enabling services

9. As mentioned in paragraph 6-8 above, when a project activity entails functions which require UNDP staff or other personnel for execution, but full-time contracting is not warranted, the project may source UNDP personnel indirectly employed for the project, who are located in the Country Office or any other UNDP location. Such service requirements, referred to as "delivery enabling services", must be budgeted within each applicable project activity or function (as described in paragraph 1 above).

10. Major activities performed by staff that may not be required full-time are listed below:

- a. **Project Supervision and Coordination** – Every project must have a project manager, who may be contracted using any appropriate contractual modality. However, for UNDP-implemented projects (DIM or full CO support to NIM), overall supervision and coordination accountability is executed by staff only. For smaller projects, UNDP may recruit staff to supervise multiple projects, in which case each project must budget its share of delivery enabling services.
- b. **Programme Coordination** – This function is provided by UNDP staff across multiple projects. Therefore, each project's share of the delivery enabling services must be included in the budget.
- c. **Policy Advice and Quality Assurance** – These services may be procured within the project through individual consultants unless any of the criteria listed in paragraph 6 above apply. If staff support is necessary, but full-time contracting under the project is not warranted, this should be considered as delivery enabling services to be sourced from existing UNDP staff and budgeted accordingly.
- d. **Administrative, Operational, and Other Shared Services** – Every project must have the staff capacity to provide project operational support, with control functions executed by staff only. In most cases, a central support mechanism for multiple projects may prove most cost effective. Services from regional or HQ staff may also become necessary for complex project procurement, recruitment, or IT needs. Again, if full-

time contracting under the project is not warranted, this should be considered as delivery enabling services to be sourced from existing UNDP staff and budgeted accordingly.

e. Project Communications, Advocacy and Funding Partner Visibility –

These services may be procured within the project through individual consultants or external companies, with a targeted and deliverable-based terms of reference. However, if a full-time project personnel is not warranted under the project, these services may be sourced from existing UNDP staff as delivery enabling services and budgeted accordingly.

11. A comprehensive project/portfolio planning exercise must be conducted annually to assess the magnitude of delivery enabling services required for the upcoming year across all projects within a Country Office. The relevant service-providing entities must be informed in order to pre-position relevant capacities. Further guidance on Planning and Paying for Delivery Enabling Services to Projects is provided here.

12. The budget for delivery enabling services can be estimated using the proforma costs plus related operating expenses and calculated based on the amount of staff/other personnel time the project anticipates will be required for implementation. The actual cost charged should be based on the actual amount of time personnel spend supporting the project.

13. Personnel who provide delivery enabling services to projects may use a high-level timesheet when the donor requires detailed documentation. Offices can modify this template to suit funding partners' needs and requirements.

14. UNDP must discuss the project budget with funding partners in advance in order to ensure transparency and a common understanding of what functions will be performed and why they are necessary, including the delivery enabling services. These deliberations are essential, especially when an individual staff not recruited directly for the project, is estimated to exceed 40 percent (e.g. 4.8 months) of his/her annual workload on the project.

15. As mentioned in paragraph 10 above, **under no circumstances should the cost of delivery enabling services be entered in a single project budget line.** They must transparently reflect in the project budget related to the activities they support.

16. Where recruitment of full-time staff/personnel under the project is not warranted or cost-effective, the project must reimburse the cost of services based on the percentage of staff time allocated directly to the project. The mechanisms for managing such reimbursement is provided in Planning and Paying for Delivery Enabling Services to Projects.

Budgeting for Indirect Costs – General Management Support (GMS)

17. Indirect or GMS costs are necessary to run the organization and support corporate structures which provide the platform for delivering programmes and projects. Indirect costs are costs and services that cannot be traced unequivocally and cost-effectively to specific activities, projects, or programmes. These costs are recovered by charging a cost recovery rate, known as a GMS fee, on all non-core resources. GMS fees are inapplicable² to TRAC and other programme resources allocated from UNDP core programme funds.

18. GMS should be budgeted in accordance with UNDP's cost recovery policy using the minimum standard GMS rates. Detailed policies on GMS and allowable rates are provided in the POPP Resource Planning and Cost Recovery.

19. Refer to POPP sections for budgeting and costing requirements of the following funding partners:

- Costing and Budgeting of projects funded by the EU is found here.
- Costing and Budgeting of projects funded by GEF is found here.

Costing and Budgeting of projects funded by GF is found here.**PETERSGATE MEDICAL CENTRE**

Privacy Notice – Direct Care (Emergencies)

~~There are occasions when intervention is necessary in order to save or protect a patient's life or to prevent them from serious immediate harm, for instance during a collapse or diabetic coma or serious injury or accident. In many of these circumstances the patient may be unconscious or too ill to communicate. In these circumstances we have an overriding duty to try to protect and treat the patient. If necessary we will share your information and possibly sensitive confidential information with other emergency healthcare services, the police or fire brigade, so that you can receive the best treatment.~~

~~The law acknowledges this and provides supporting legal justifications.~~

~~Individuals have the right to make pre-determined decisions about the type and extent of care they will receive should they fall ill in the future, these are known as "Advance Directives". If lodged in your records these will normally be honoured despite the observations in the first paragraph.~~

Data Controller contact details	Petersgate Medical Centre, 99 Amersall Road, Doncaster, DN5 9PQ Tel: 01302 390490
Data Protection Officer Contact details	The Practice Data Protection Officer (DPO) is Caroline Million, Independent Data Protection Officer. Caroline Million, Caroline.million@outlook.com 07912 975522
Purpose of the processing	Doctors have a professional responsibility to share data in emergencies to protect their patients or other persons. Often in emergency situations the patient is unable to provide consent.
Lawful basis for processing	This is a Direct Care purpose. There is a specific legal justification; <i>Article 6(1)(d) "processing is necessary to protect the vital interests of the data subject or of another natural person"</i> And <i>Article 9(2)(c) "processing is necessary to protect the vital interests of the data subject or of another natural person where the data</i>

² GMS fees are inapplicable to 'Special Volunteers Fund'

	subject is physically or legally incapable of giving consent” Or alternatively Article 9(2)(h) “necessary for the purposes of preventative or occupational medicine for the assessment of the working capacity of the employee, medical diagnosis, the provision of health or social care or treatment or the management of health or social care systems and services...” We will also recognise your rights established under UK case law collectively known as the “Common Law Duty of Confidentiality”²
Recipient or categories of recipients of the shared data	The data will be shared with Healthcare professionals and other workers in emergency and out of hours services and at local hospitals, diagnostic and treatment centres. (if preferred list actual named services)
Rights to object	You have the right to object to some or all of the information being shared with the recipients. Contact the Data Controller or the practice. You also have the right to have an “Advance Directive” placed in your records and brought to the attention of relevant healthcare workers or staff.
Right to access and correct	You have the right to access the data that is being shared and have any inaccuracies corrected. There is no right to have accurate medical records deleted except when ordered by a court of Law. If we share or process your data in an emergency when you have not been able to consent, we will notify you at the earliest opportunity.
Retention period	The data will be retained in line with the law and national guidance
Right to Complain.	You have the right to complain to the Information Commissioner’s Office, you can use this link https://ico.org.uk/global/contact-us/ or calling their helpline Tel: 0303 123 1113 (local rate) or 01625 545 745 (national rate) There are National Offices for Scotland, Northern Ireland and Wales, (see ICO website)

* “Common Law Duty of Confidentiality”, common law is not written out in one document like an Act of Parliament. It is a form of law based on previous court cases decided by judges; hence, it is also referred to as ‘judge-made’ or case law. The law is applied by reference to those previous cases, so common law is also said to be based on precedent.

The general position is that if information is given in circumstances where it is expected that a duty of confidence applies, that information cannot normally be disclosed without the information provider’s consent.

In practice, this means that all patient information, whether held on paper, computer, visually or audio recorded, or held in the memory of the professional, must not normally be disclosed without the consent of the patient. It is irrelevant how old the patient is or what the state of their mental health is; the duty still applies.

Three circumstances making disclosure of confidential information lawful are:

- where the individual to whom the information relates has consented;
- where disclosure is in the public interest; and
- where there is a legal duty to do so, for example a court order.
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